

Title: SRHR Knowledge mapping online survey report

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Executive Summary

Youth Policy Forum (YPF) conducted a survey targeting the university students to understand their knowledge and awareness regarding Sexual and Reproductive Health and Rights (SRHR). This survey was conducted through Google form and primarily disseminated through YPF social media handles. Our aim was to map the existing knowledge across critical themes such as basic SRHR knowledge, awareness of STDs and HIV/AIDS, gender equality, violence and safety, skills for health and well-being, and the intersection of human rights and sexuality.

579 people, comprising a balanced representation of genders, participated in our survey. Key findings revealed that friends and family were significant sources of SRHR information. While respondents displayed a solid basic knowledge of SRHR, there were areas for improvement in communication and negotiation skills for health and well-being. The survey highlighted the participants' understanding of consent and healthy relationships, shedding light on existing misconceptions and the need for comprehensive education.

Despite our limitations, this survey offers a foundational understanding of SRHR awareness and attitudes among university youth. This report concludes with recommendations for future research, emphasizing longitudinal studies, demographic diversification, qualitative exploration, and targeted interventions to foster an open conversation around SRHR topics.

About YPF

Youth Policy Forum is a platform of young Bangladeshi people from all over the world who have an interest in nation-building and want to contribute through effective policy formulation and implementation. By creating a pool of young intellectual minds, YPF intends to establish a comprehensive support system that will help the government, non-government, and global policymakers to formulate and adopt more inclusive policies.

Background of the Survey

The Youth Policy Forum (YPF) is a dynamic coalition member in the Right Here Right Now 2 (RHRN 2) multi-stakeholder coalition. The second phase of RHRN in Bangladesh is being spearheaded by BRAC, with Naripokkho, RHSTEP, Nagorik Udyog, and Oboyob serving as coalition partners alongside Wreetu, another youth-led coalition partner.

The fundamental objective of the RHRN2 partnership is to ensure that young people know about their reproductive health within equitable and gender-just societies. The driving force behind our partnership lies in the active involvement of young individuals at the forefront of this transformative initiative.

To achieve our vision, we are determined to harness the inherent potential of youth and garner widespread public support for reproductive health matters. Our core strategy centers around lobbying and advocacy efforts to effect positive change by developing and strengthening policies and laws.

As a youth-led coalition partner, YPF focuses on university interventions, digital campaigns, and policy advocacy to contribute to informed and empowered communities. This aligns with RHRN2's broader goals of leveraging youth influence for enhanced SRHR support, policy impact, and societal strengthening.

In the pursuit of finding a comprehensive understanding of Sexual and Reproductive Health and Rights (SRHR) among university youth, YPF initiated this survey aimed at mapping the existing knowledge of young individuals. The online survey was conducted through Google Form and disseminated through various social media platforms, including Facebook, Twitter, Instagram, and LinkedIn.

Given that young people are crucial in forming society's attitudes and conventions, we made an effort to explore the subtleties of their comprehension of SRHR issues. The survey attempted to paint an in-depth understanding of the youth's perceptions and knowledge landscape by focusing on important themes like basic SRHR knowledge, awareness of STDs and HIV/AIDS, Gender Equality, Violence and Safety, Skills for Health and Well-Being, and the intersection of Human Rights and Sexuality.

Characteristics of the Respondents

Summary of Information

Gender	Total= 579 Male= 289 Female= 262 Transgender= 7 Different gender identity= 15 Prefer not to answer= 6
Age	Total= 579 Under 18= 42 18 to 20= 161 21 to 25= 316 25 or older= 56 Prefer not to answer= 4
Institution	Government= 438 Private= 141
Current University Year	College Student=68 1 st Year=153 2 nd Year=85 3 rd Year=79 4 th Year=87 Post Graduate=54 Those who do not belong to any of the category described above=53
Current Residence	Inside Dhaka:115 Outside Dhaka:464
Marital Status	Married-56 Unmarried-517 Divorced-3

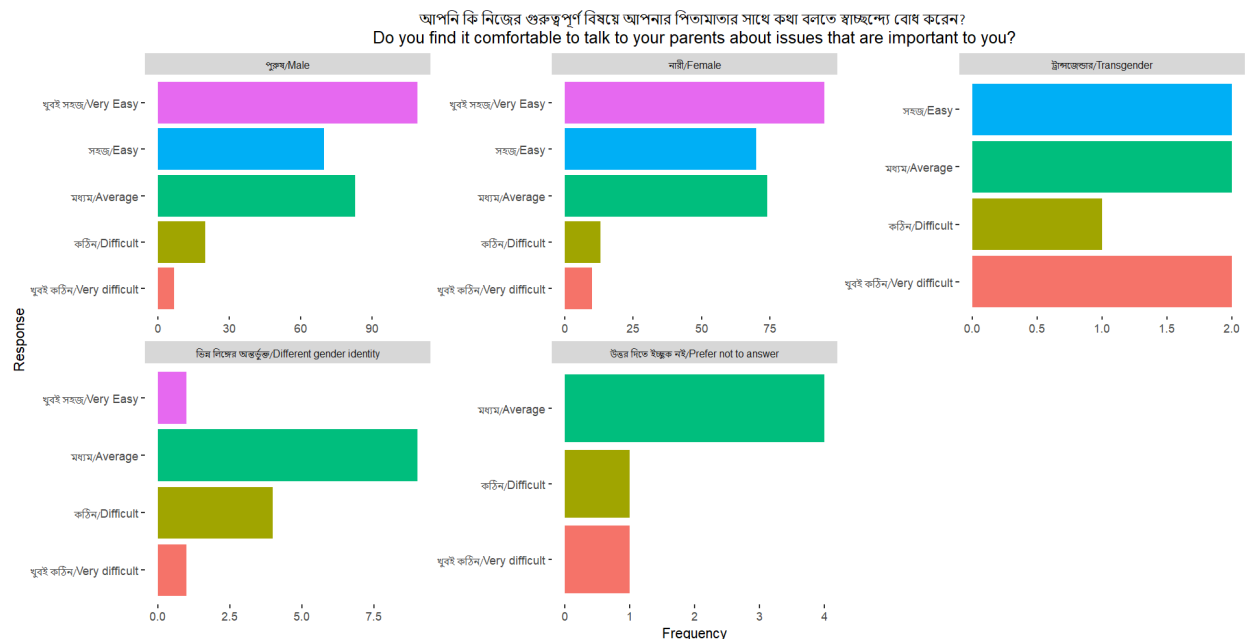
	Do not want to specify-3
Residential Status	Lives with their parents- 502 Does not live with their parents-77
Household Member Count	One member-2 Two members-20 Three members-125 Four or more members-432

Comfortability of discussing important issues with parents

Majority (89.64%) of the participants responded that they are comfortable talking about issues that are important to them.

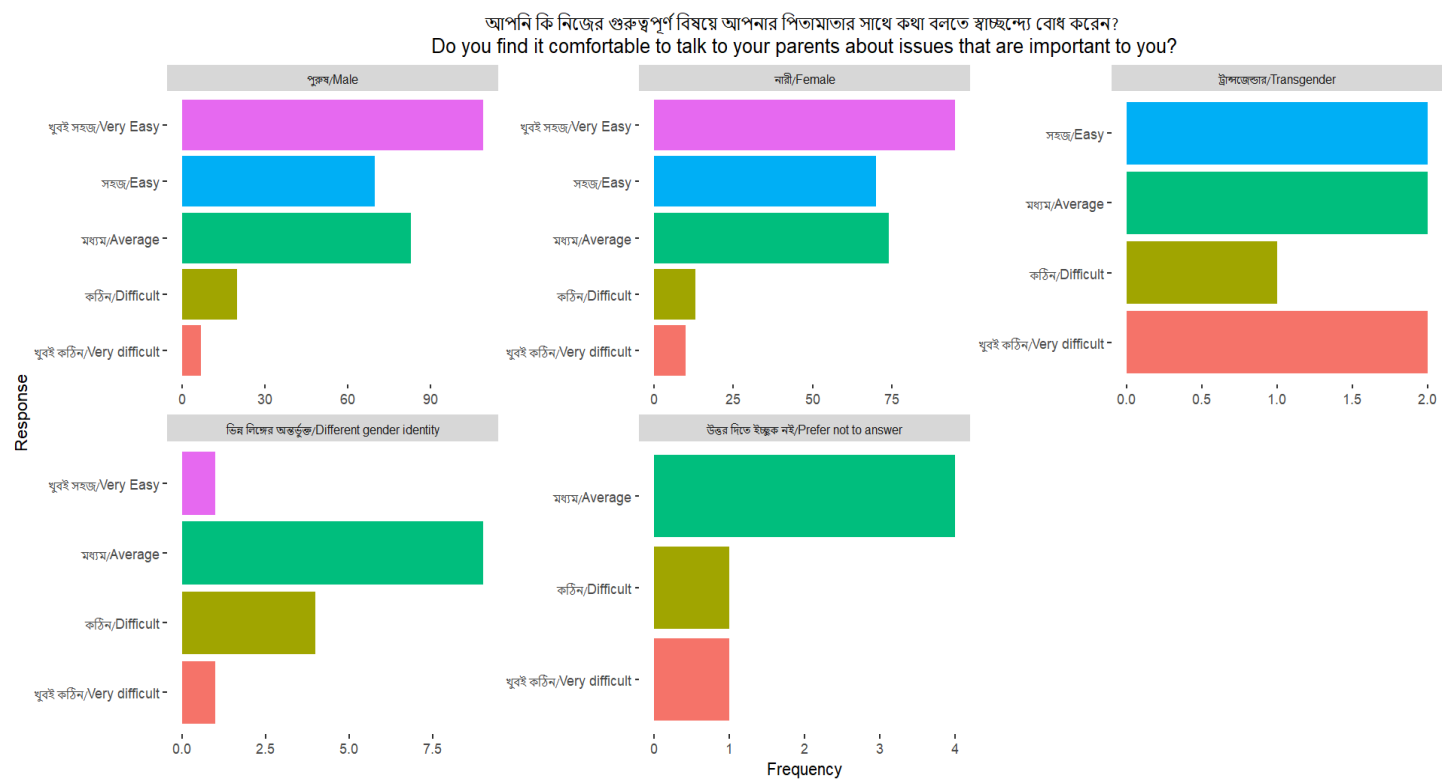
It is notable that the majority in almost all age groups find it easy or very easy to talk to their parents or siblings about important issues.

Except for those who are 25 or above, the majority in this age group described it as ‘Average’. This could potentially refer to a more positive parenting in the recent past.



It is notable that while most male and female gendered respondents find it easy to talk to their parents, a significant portion of transgender participants felt it is ‘Difficult’ or ‘Very Difficult’ to do so.

Majority among those who identify as ‘Different Gender’, and those who ‘Preferred not to Reveal’ their gender described their comfortability as ‘Average’



Our survey found no significant difference in their response to this question based on the type of institution (Government or Private) they work in.

Relationship status of parents

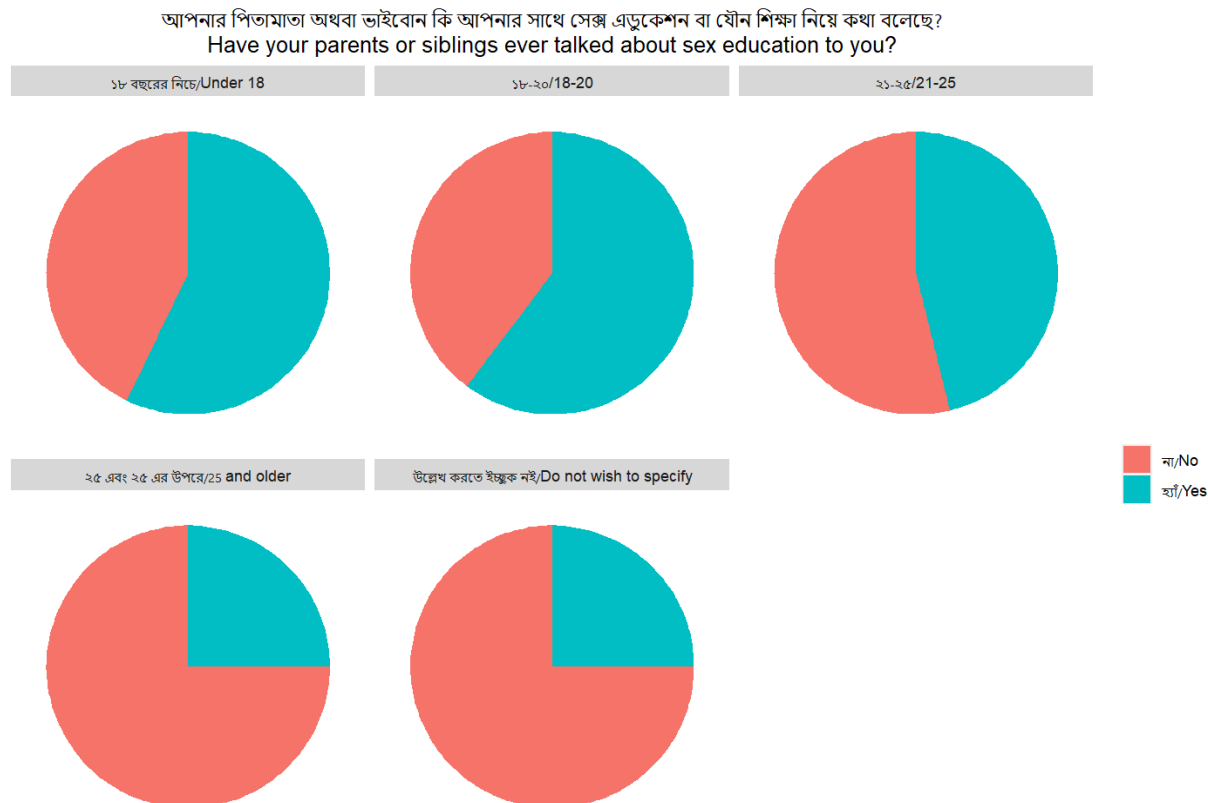
Majority (90%) of the participants think that their parents' relationship is healthy and respectful.

Household member count

Majority (74.6%) of the participants are currently living in a household with 4 or more members.

SRHR discussion with family members

Although the majority (51.3%) replied that their parents or siblings never talked about sex education with them, it's not significant enough to come to any conclusion.



It's noteworthy that the majority of the respondents, aged 20 or below, have been talked to by their parents about sex education.

Whereas majority of the participants aged 21 and above, and those who did not wish to disclose their age mentioned that their parents or siblings never talked about sex education to them. This discrepancy shows a potential shift in parental communication as the respondents transition into adulthood.

Majority of the female respondents replied that they conversed with their parents and siblings about sex education. While the majority participants of other genders replied in negative. This might be due to the fact that females typically have closer relationships with their mothers.

Findings

Sources of information on SHRH within family and social circle

According to the majority of the respondents, 'Friends and Family' are the most common source of information related to SRHR. This is contrary to the belief that SRHR is a taboo subject in our society and is mostly avoided discussions about. However, this might be because of the distribution of respondents' demographics which mostly comprises educated youths.

Key Findings

Majority (52.3%) of the participants first learnt about puberty from their 'Friends', while 'Books/Magazine/TV/Movies', and 'Teachers' come 2nd and 3rd respectively.

The three major sources of information on Puberty are "Friends and Family", 'Other Mediums', and 'Professionals' respectively.

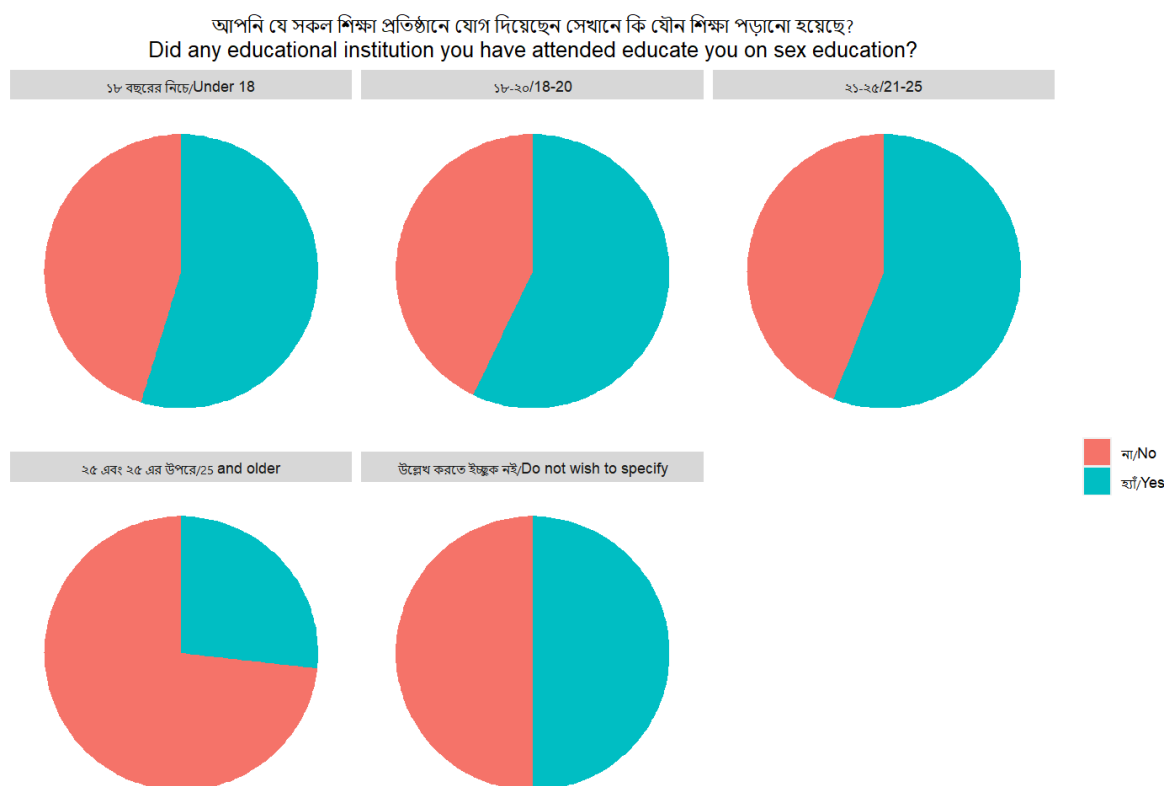
Majority (27.8%) of the participants first learnt about the sexual and reproductive system of men and women from their 'Friends', while 'Books/Magazine/TV/Movies', and 'Teachers' came 2nd and 3rd respectively.

The three major sources of information on the sexual and reproductive system of men and women are 'Friends and Family', 'Other Mediums', and 'Professionals' respectively.

An overwhelming majority (53.54%) of the participants were introduced to the concepts of consent and respect in a relationship by their parents.

The three major source of information on the concepts of consent and respect in a relationship are "Friends and Family", 'Other Mediums', and 'Professionals' respectively.

Majority (53.4%) of the participants were educated on 'Sex Education' at the school they attended.



Majority of the participants aged 25 or below were educated on ‘Sex Education’ at the school they attended.

Majority of the respondents who are ‘aged 25 and above’ however responded that they were not. This shows that there most probably has been a potential shift in the curriculum or in the mentality of the teachers in the recent years.

The majority of both male and female respondents indicated that they received ‘Sex Education’ at their schools. However, most participants of other genders responded negatively. This suggests that individuals of genders other than male and female are more likely to have a knowledge gap in SRHR, underscoring the need to focus more on these groups

The majority of participants from government institutions responded ‘Yes’, while the majority from private institutions responded ‘No’. This discrepancy might be due to the possibility that private institutions may not provide adequate training to their teachers about SRHR, or it could indicate the need to update their overall curriculum.

Programmatic recommendation:

From our findings, it is evident that there are several areas where improvements can be implemented in providing Sexual and Reproductive Health and Rights (SRHR) services. Such as-

- **Creating diverse educational resources:** The survey data shows that friends and family are the primary source of information on puberty and the sexual and reproductive system. This suggests a need for more diverse and accessible educational resources that can provide accurate and comprehensive information on these topics. We need to develop educational resources on Sexual and Reproductive Health and Rights (SRHR) that are age-appropriate. It is vital to ensure that these resources are culturally sensitive and inclusive, respecting and acknowledging different gender identities.

(Aligns with **Pathway 1: Information and Education**. Specifically, the development of gamified SRHR content on the web portal, including age-appropriate and culturally sensitive resources on puberty and the sexual and reproductive system.)

- **Comprehensive Sex Education programs:** The fact that a majority of participants were educated on sex education at school indicates the potential effectiveness of comprehensive sex education programs. However, the data also shows a knowledge gap among participants aged 25 and above, highlighting the need for these programs to be ongoing and inclusive of all age groups. There is a need to implement comprehensive sex education programs that encompass a wide range of topics. These should include the changes that occur during puberty, an understanding of the sexual and reproductive system, and the importance of consent and respect in all relationships. Such programs can equip individuals with the knowledge and skills they need to navigate their sexual and reproductive health responsibly.

(Aligns with **Pathway 1: Information and Education**.)

- **Parental involvement events:** Given that a significant majority of participants were introduced to the concepts of consent and respect in relationships by their parents, parental involvement events could be an effective way to ensure that these crucial topics are adequately addressed. Organizing parental involvement events that engage parents in meaningful conversations about crucial topics can be potent. These topics should include puberty, sexual and reproductive health, the importance of consent, setting boundaries, and fostering respect in relationships. Such events can encourage parents to effectively guide their children who navigate their emotions through these complex issues.

(Aligns with **Pathway 1: Information and Education**.)

- **Promoting peer education:** Considering that the majority of the participants in the survey acquired firsthand knowledge from their friends, it's crucial to ensure this information is accurate and comprehensive. We need to create safe spaces where peers can discuss SRHR topics among themselves. This approach can foster a supportive and informed community, enhancing the overall understanding of SRHR from a young age.

(Aligns with **Pathway 1: Information and Education.**)

- **Community workshops and forums:** Our survey data shows a disparity in sex education based on the type of institution, with participants from private institutions less likely to have received sex education. Community workshops and forums could help address this disparity by providing an additional source of information outside of the school environment. We need to establish community workshops and forums to encourage open dialogue about Sexual and Reproductive Health and Rights (SRHR). These platforms can help minimize the stigma associated with SRHR issues, fostering a more inclusive and understanding community. By promoting open discussions, we can address misconceptions, increase awareness, and ultimately improve the overall understanding of SRHR.

(Aligns with **Pathway 1: Information and Education.**)

- **Regular Monitoring and Evaluation of the programs:** The survey data shows differences in sex education based on age and gender, indicating the need for regular monitoring and evaluation to ensure that these programs are reaching all demographics effectively. Implementing regular monitoring and evaluation of the programs by collecting feedback from key stakeholders such as students, parents, and teachers is crucial. This feedback can help assess the effectiveness of the programs and identify areas for improvement. By continuously refining these programs based on feedback, we can ensure the resources remain relevant, effective, and responsive to the needs of the community.

(Aligns with all Pathways, with a particular emphasis on **Pathway 1: Information and Education and Pathway 3: Policy Advocacy**)

Understanding the meaning of consent and healthy relationships

Contrary to the popular belief that people may not fully understand what consent is and how a healthy relationship should function, our findings suggest that most respondents have a good understanding of these concepts or at least the 'claim' that they do. However, from the insights we gained from the survey, it is concerning as to how little they actually know about 'consent'.

To further explore this issue we asked our participants a specific query, “Does any kind of physical reaction (arousal) in a person automatically imply consent?” The goal behind asking this question was to gauge the depth of respondents’ level of understanding on the nuanced aspects of consent. Despite their claims of understanding, the survey did reveal a potential gap in knowledge, prompting an examination on the actual level of awareness on this matter. Our analysis shows that, there is a presence of following:

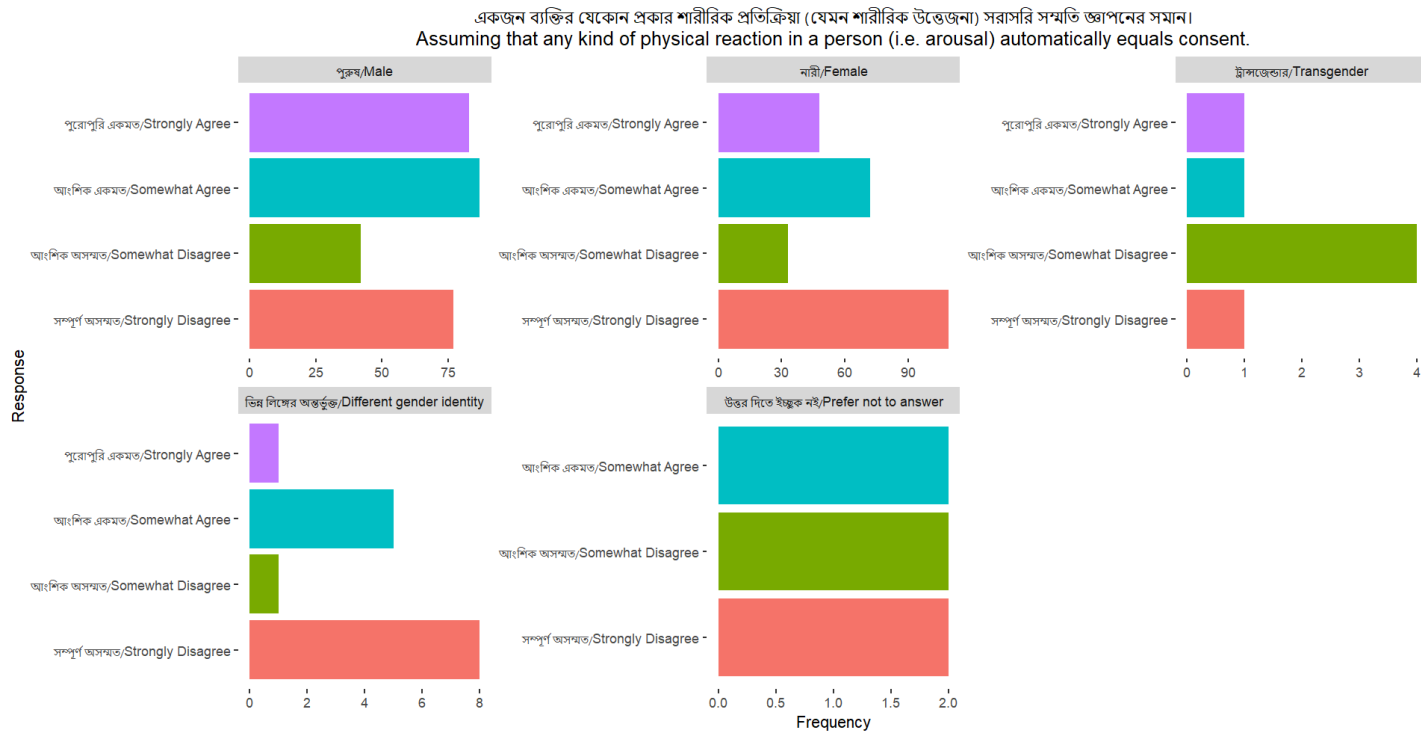
- **Misconception of consent:** Assuming consent based solely on physical reactions oversimplifies the complex nature of human interactions. Consent is not merely about physical responses but involves clear, verbal, and ongoing communication between all parties involved.s.
- **Risk of non-consensual acts:** Relying on physical arousal as an indicator of consent can increase the risk of non-consensual acts. This mindset neglects the necessity of clear communication and mutual agreement, which are fundamental to consent.
- **Understanding non-verbal cues:** While changes in body language, facial expressions, and gestures can provide some insight into a person’s feelings, they should not be accepted as consent. Communication should always be explicit and verbal. It’s also important to discuss the concept of “withdrawal of consent,” emphasizing that consent is not a one-time agreement but an ongoing process requiring continuous affirmation.
- **Failure to identify individual reactions:** Individuals express themselves and experience arousal differently. Therefore, physical reactions cannot be used as a universal benchmark for consent, as they vary based on each individual’s comfort level and boundaries
- **Violating one's autonomy:** Assuming consent based on someone’s physical arousal disregards the autonomy and agency of an individual. This mindset suggests that one person can make assumptions about another person’s desires without consulting them, which directly undermines the principles of respecting one’s choices..

The fact that 51.8% of the respondents ‘strongly agree’ or ‘somewhat agree’ that any kind of physical reaction (arousal) in a person automatically means consent underscores a significant gap in the actual understanding of consent. This highlights the dire need for comprehensive education on consent to educate the masses.

Key Findings

- Most (61.66%) of the respondents are not in a relationship
- Majority (81.3%) of the participants know what consent means

- Majority (67.7%) of the participants strongly disagree with the statement that ‘You can occasionally hit your partner because both of you are committed’
- Majority (34%) of the participants strongly disagree with the notion that any kind of physical reaction in a person automatically equals consent. However, an alarming number of respondents strongly or somewhat agree with this.
- Majority of the participants below the age of 21, and those who did not disclose their age strongly or somewhat agree with the notion that any kind of physical reaction in a person automatically equals consent. However, the majority of the respondents who are 21 or older strongly disagree with this.



- Majority of the male participants somewhat agree that any kind of physical reaction in a person automatically equals consent; in contrast, most transgender respondents ‘Somewhat disagree’ with this statement. Majority of the female and ‘different gender identity’ respondents however ‘strongly disagree’. This shows that there is a major lack of understanding of consent among the participants.
- Although the majority of both Government and Private institution students disagrees with this, a significant portion of both strongly or somewhat agrees to this as well.
- Majority(69.26%) of the Participants strongly disagree with the statement that a girl’s clothing choices suggest she is seeking or open to sexual activity.
- Majority (40.76%) of the participants strongly agree with the statement that if an individual isn't resisting or vocalizing objections, it can be assumed that they are agreeing

for sexual relationships. For example, if two people are engaging in intimate actions and neither of them expressing discomfort or disagreement, it might be referred to as consent.

- Majority of the respondents in all the age groups agree with the statement that if an individual isn't resisting or vocalizing objections, it can be assumed that they are agreeing for sexual relationships. Most of the male and female respondents also strongly agree with this statement.

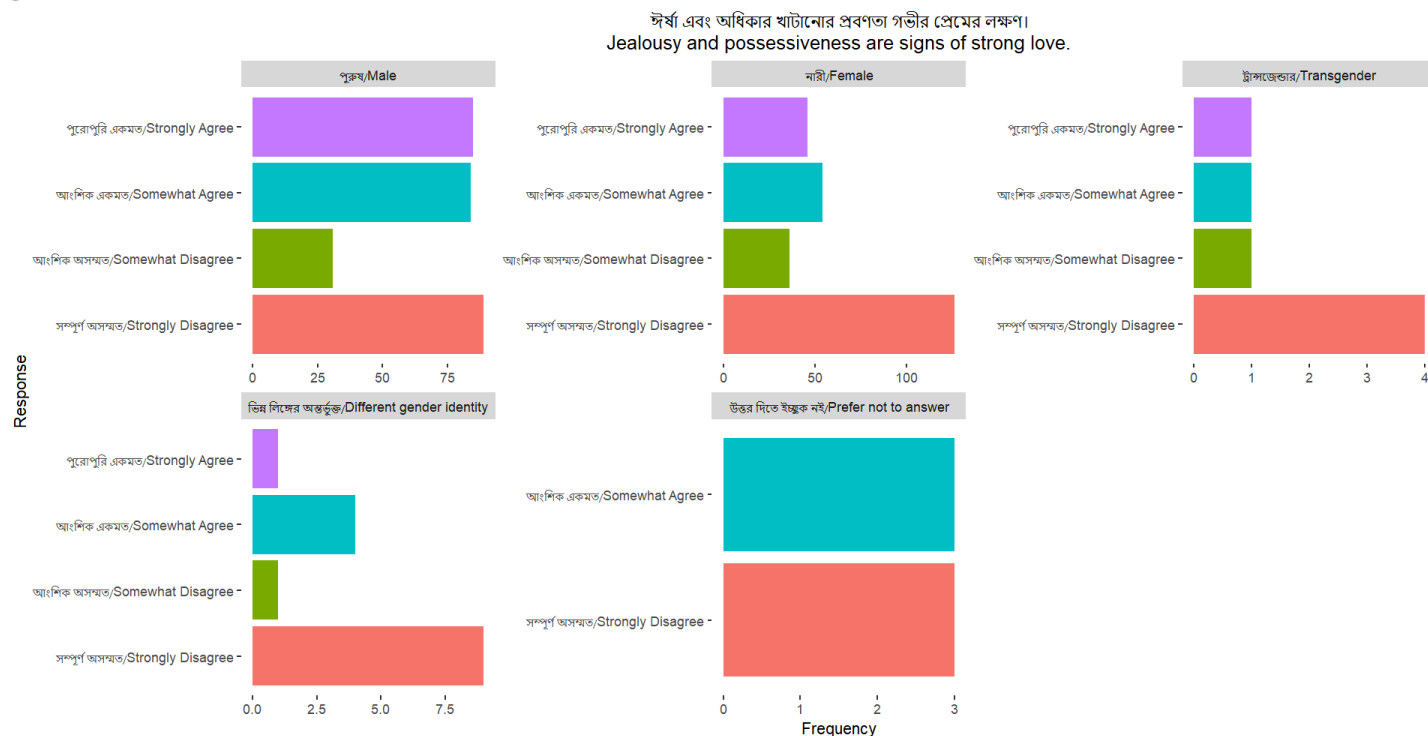
This data shows the nuanced view of non-verbal communication in any intimate relationship. It focuses on the need for explicit communication and how multifaceted the process of consent can be. Respondents having Different Gender Identities however strongly disagree with this. Transgender people and those who did not disclose their gender are almost equally divided between agree and disagree.

Majority of the participants 'Strongly Agree' to this statement that engaging in sexual activity with someone who is unconscious or under the influence of drugs is considered rape, as they are unable to provide informed and voluntary consent. i.e. if someone at a party has consumed a significant amount of alcohol and has passed out, any sexual activity with them without their explicit and conscious agreement would be classified as rape.

- Majority of the participants conform to the notion that in situations where a person has a disability that prevents them from giving consent, a responsible guardian or caregiver may need to take decisions on their behalf. For example, if an individual with severe communication disability is in a romantic relationship, their legal guardian might be involved in determining whether certain intimate activities are appropriate and consensual for that person.

However, this approach needs to be taken carefully to avoid any potential risk of abuse or infringements on the rights of the individuals with disability. Our goal will be to focus on empowering individuals with disabilities to communicate their preferences and to be able to participate in their decision making process.

- Majority (89.1%) of the participants Strongly Agree to this statement that NO means no, and there is no further explanation needed if a person does not want to have sex.
- Majority (39.89%) of the participants strongly disagree with the statement that jealousy and possessiveness are signs of strong love.
- Majority of the respondents below the age of 18 strongly agree with the statement that jealousy and possessiveness are signs of strong love. Majority of the participants older than 21, however, strongly disagree with this. Respondents between the age of 18-20 are, however, almost evenly split over this. It is evident that young minds often confuse jealousy and possessiveness with love, mistakenly attributing intensity to these toxic traits, unaware that love thrives in trust and respect, not in control and insecurity. As they grow, understand that genuine affection fosters freedom.



- Majority of the respondents from all genders disagree with the statement that jealousy and possessiveness are signs of strong love.
- Majority of both Private and Government job holders disagree with this statement that jealousy and possessiveness are signs of strong love.
- Majority (71.5 %) of the participants strongly disagree with this statement -‘It is acceptable to abuse someone through any form of abuse (verbal, sexual, mental, financial or digital) when you are in a relationship. For example, if a partner constantly humiliates another partner through hurtful comments and insults, that will be acceptable in certain contexts.’
- It shows that the majority of the participants are aware that abuse can go beyond physical harm and can affect the well-being of the individuals who go through such forms of abuse.
- Majority of the participants are not eligible to answer the question most possibly for being sexually inactive, Excluding these ‘Not Applicable’ participants, almost 73.47% of the rest of the respondents or their partners use contraceptives either during or following a sexual relationship
- Excluding the ‘Not Applicable’ majority, around 70.8% of the rest of the respondents use condoms as their go-to contraceptives.
- Majority (79.79%) of the participants did not face a situation where they were forced to have sexual relationships with a family member, friend or stranger.
- Excluding the ‘Not Applicable’ majority, Exactly Half of the respondents talked with family and/or close friends about the incident where they were forced to have sexual relationships with a family member, friend or stranger.,while the other half did not.

However it is noteworthy that social stigma or maintaining privacy might affect the survey responses regarding the disclosure, keeping in mind the sensitive nature of this question.

Programmatic recommendation:

Our survey reveals a concerning lack of understanding about consent, indicating an urgent need for comprehensive and targeted consent education. Here are some key areas that need to be addressed:

- **Misconception on non-verbal consent:** A significant percentage of respondents, especially those below the age of 21, somewhat agree that physical reactions equate to consent. This misconception needs to be addressed to prevent potential abuse in relationships. Education should emphasize that consent is not merely about physical reactions but involves clear, verbal, and ongoing communication.

(Aligns with **Pathway 1: Information and Education**)

- **Identifying toxic traits:** Our survey shows that younger participants often mistake jealousy and possessiveness as signs of strong love. Initiatives are needed to educate them about healthy relationship dynamics and to help them distinguish between love and control.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Abuse disclosure:** The split responses on forced sexual relationships highlight potential challenges. Exploring anonymous reporting mechanisms and ensuring survivors' confidentiality can address fears of defamation and privacy concerns. This approach can encourage more victims to come forward and seek help.

(Aligns with **Pathway 2: Public Support**)

- **Disability inclusive guidelines :** There is a need for disability-inclusive guidelines that empower people with disabilities. The goal is to prevent potential abuses or violation of rights and to ensure that everyone, regardless of their abilities, has access to comprehensive education on consent and healthy relationships. This not only ensures the rights of people with disability but also aids to foster a culture of respect and understanding for everyone.

(Aligns with **Pathway 1: Information and Education** and **Pathway 3: Policy Advocacy**)

- **Communication on "No means no":** The majority strongly agrees with the statement "No means no," which is encouraging. This understanding needs to be reinforced through frequent campaigns focusing on the importance of respecting boundaries. The campaigns

can use diverse social media platforms, workshops and educational institutions to continuously reinforce the basic understanding of mutual respect and consent.

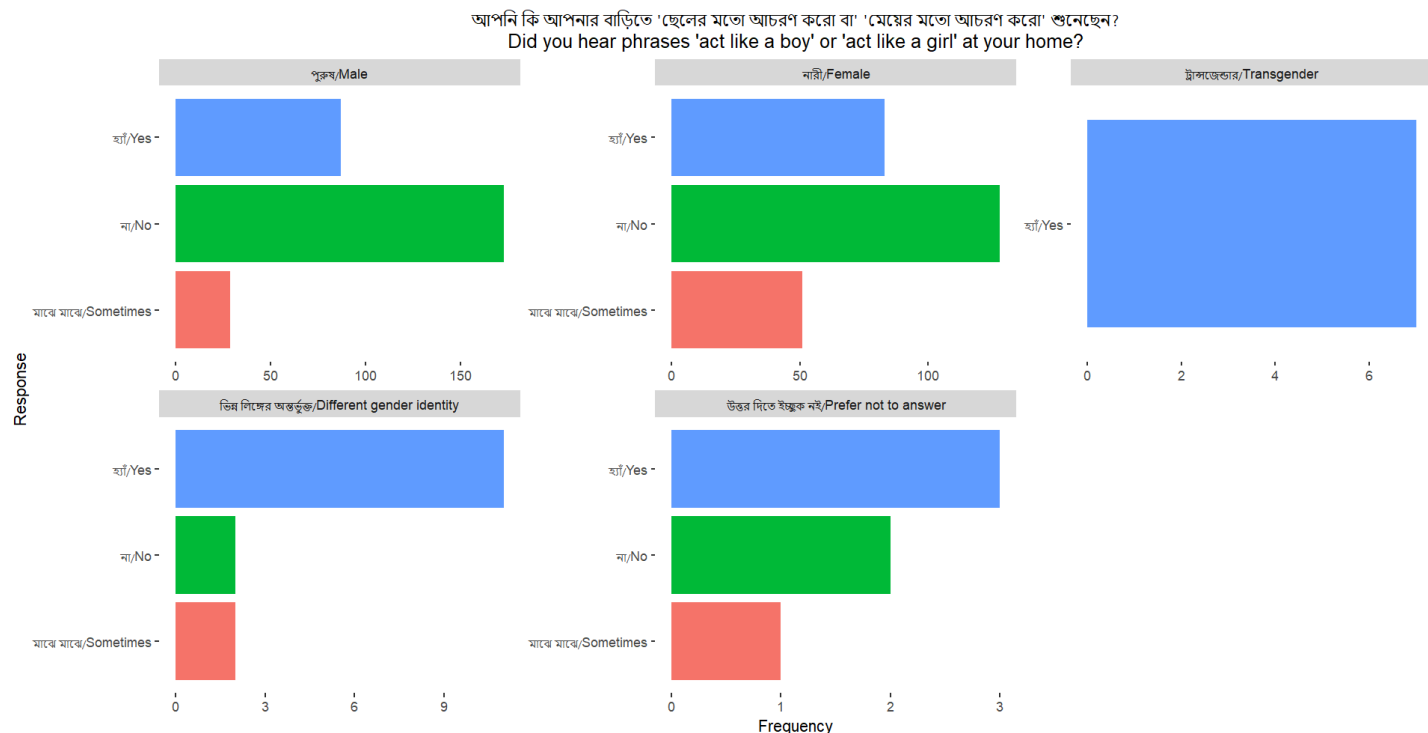
(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

Gender equality, stereotypes and bias

The results from this showcase that the respondents do have a fairly good grasp on the concepts of gender equality, stereotypes, and bias. However, from the responses it is clear that there exist widespread gender inequality, stereotypes and bias towards any gender other than male. This signifies the need for a more thorough approach in eradicating gender injustice in our society.

Key Findings

Majority (52.68%) of the respondents did not hear phrases like “act like a boy” or “act like a girl” at their home from their family



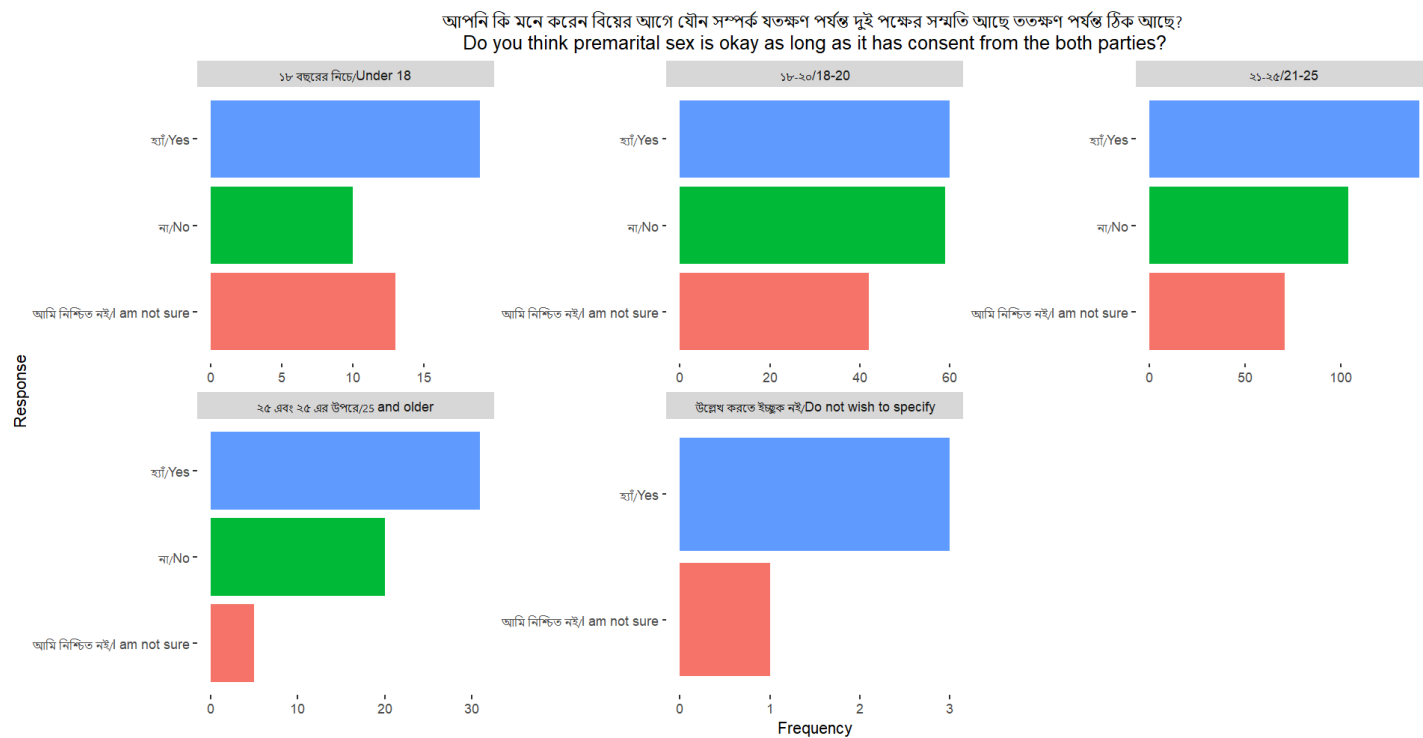
Although the majority of the other gender responded that they did hear phrases like “act like a boy” or “act like a girl” at their home from their family, but of the male and female gender did not hear about such phrases.

Majority (81.86%) of the participants think in most Bangladeshi marriages, husband and/or husband’s family has more dominance in the decision making process.

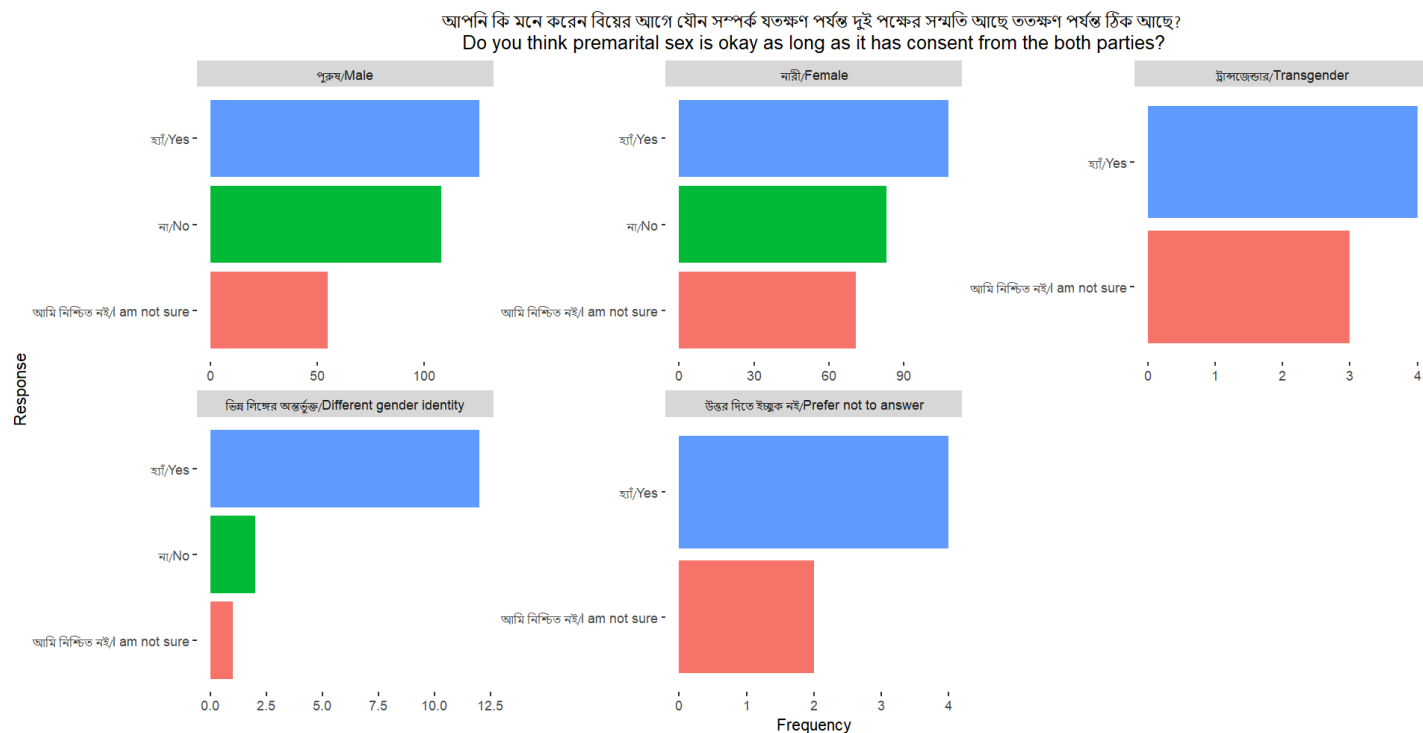
Majority (68%) of the participants strongly agree with this statement that signs of an unhealthy partnership include feeling unable to speak up for yourself and experiencing intimidation from your partner. This data emphasizes the importance of open communication and recognizing potential threats in a relationship.

Majority (58.2%) of the participants strongly agree with this statement- ‘Some of the textbook chapters in Bangladesh depict men in leadership roles and women in passive or domestic roles, this visual representation can contribute to gender bias. Also, if the language used in textbooks assumes certain professions belong to men (i.e. firemen instead of firefighters), it reinforces linguistic sexism.’ It’s noteworthy that the rest of the respondents Somewhat agree with this statement.

Majority (43.86%) of the participants think premarital sex is okay as long as it has consent from the both parties. This data indicates respecting individual choices and practicing open communication in intimate relationships.



Majority of the participants from all the age groups think premarital sex is okay as long as it has consent from the both parties.



Most individuals across gender categories believe that premarital sex is acceptable, provided there is mutual consent between both parties.

Programmatic recommendation:

The outcomes of our survey reveals a concerning lack of understanding about gender equality, stereotypes, and bias. This indicates an urgent need for comprehensive and targeted education. Here are some key areas that need to be addressed:

- Implementing relationship programs:** With 68% of participants strongly agreeing that signs of an unhealthy partnership include feeling unable to speak up for yourself and experiencing intimidation from your partner, there's a clear need for programs that promote open communication in relationships. Workshops could be organized to teach individuals how to express their feelings and needs effectively and respectfully. Furthermore, we can introduce relationship programs that help individuals recognize the signs of unhealthy partnerships, such as intimidation, isolation, disrespect, unresolved anger issues, unaddressed past incidents of abuse etc.. These programs can equip individuals with the knowledge and skills to identify and address toxic behaviors in any relationship.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Addressing Dominance in Marriages:** As 81.86% of participants think that in most Bangladeshi marriages, the husband and/or husband's family has more dominance in the decision-making process, it's important to address this issue. Workshops could be organized to promote equality and shared decision-making in marriages. These initiatives can introduce the participants to practical tools and strategies to practice active listening, healthy decision making process and awareness on equitable relationships.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Educating about Consent in Premarital Relationships:** Our survey shows that 43.86% of participants think premarital sex is okay as long as there is consent from both parties. This highlights the importance of educating individuals about the concept of consent, especially in the context of premarital relationships. Awareness campaigns could be launched to emphasize that consent must be clear, enthusiastic, and ongoing.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Gender sensitization events:** Although the majority of male and female participants did not hear phrases like "act like a boy" or "act like a girl" at home, the majority of other genders did. This suggests a need for initiatives that empower individuals of other genders and challenge traditional gender norms. Gender sensitization events could be organized to promote a more inclusive understanding of gender identities. These events can highlight the impact of such phrases in reinforcing gender stereotypes and promote a more inclusive understanding of gender identities.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Addressing Gender Stereotypes in Education:** Given that 58.2% of participants strongly agree that textbooks in Bangladesh often depict men in leadership roles and women in passive or domestic roles, it's crucial to address these gender stereotypes in educational materials. We recommend reviewing and revising textbooks to ensure they promote gender equality and do not reinforce linguistic sexism.

(Aligns with **Pathway 1: Information and Education** and **Pathway 3: Policy Advocacy**)

- **Age appropriate awareness campaigns:** Launch age-appropriate awareness campaigns to educate individuals about consent and healthy relationships. These campaigns can be tailored to the unique needs and understanding of different age groups, ensuring that the information is relevant and accessible.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Pre and post assessments of the participants:** Conduct pre and post assessments of event participants to measure changes in their knowledge and attitudes. These assessments can provide valuable insights into the effectiveness of these initiatives and help identify areas for improvement.

(Aligns with all Pathways, emphasizing the importance of evaluation in **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

Basic knowledge of SRHR and knowledge on STDs

The responses of the participants of our survey indicates that most of the respondents are well versed with the basic knowledge of SRHR and knowledge on STDs.

Key Findings

Majority (97.4%) of the participants agree with this statement that everyone has the right to marry (or not to marry) and to start a family with free and full consent and the right to decide on the number and spacing of one's children.

Majority (72.71%) of the respondents believe that they should approach a doctor if they need information about contraceptives, pregnancy, STDs or any sexual health related issues.

Skills for Health and Well-being (communication, refusal and negotiation skills)

This survey shows an alarming situation regarding the mental health of the respondents. Majority of the participants feel anxious or depressed at times. Most of the respondents are not used to hearing 'no' as an answer and sometimes feel angry as a result.

Also it is noteworthy that the respondents are conflicted over what to do if they find themselves in an abusive relationship.

Key Findings

Majority (59.41%) believe effective communication, listening to your partner, being able to speak for yourself, taking the stand for yourself without the fear of judgment, mutual trust and respect, and setting personal space and boundaries are the main aspects of a healthy relationship

Most (55.44%) of the respondents shared that they feel anxious or depressed often

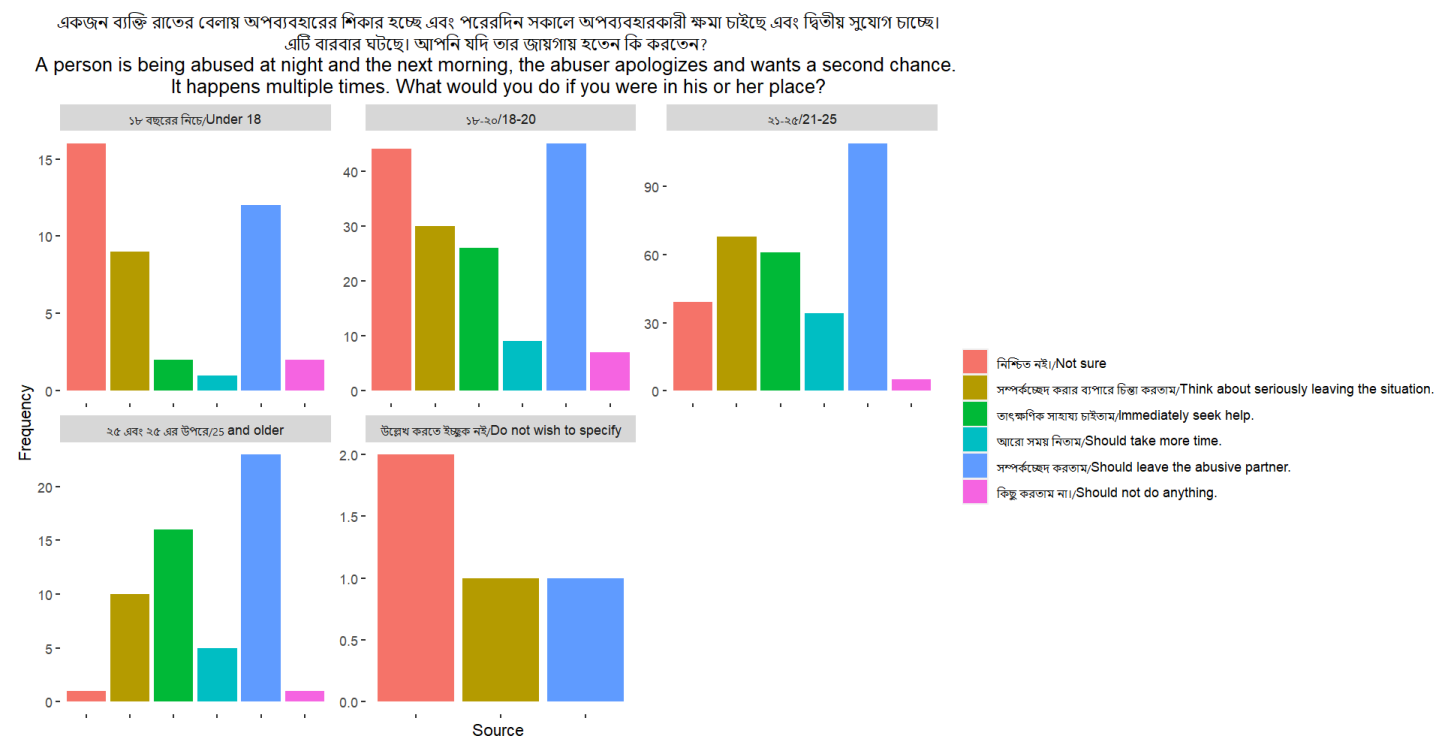
Majority (53.88%) responded that when someone says NO to them it sometimes make them angry or upset

Excluding the 'Not applicable' majority, 75.6% of the rest responded that they can effectively communicate with your partner about sexual healthy boundaries in regards to contraception, sexual consent, pregnancy and STDs

We asked the respondents what they would do if they were in his or her place? given the scenario below

Scenario: A person is being abused at night and the next morning, the abuser apologizes and wants a second chance. It happens multiple times.

Majority (32.81%) of the respondents believe that they should leave the abusive partner

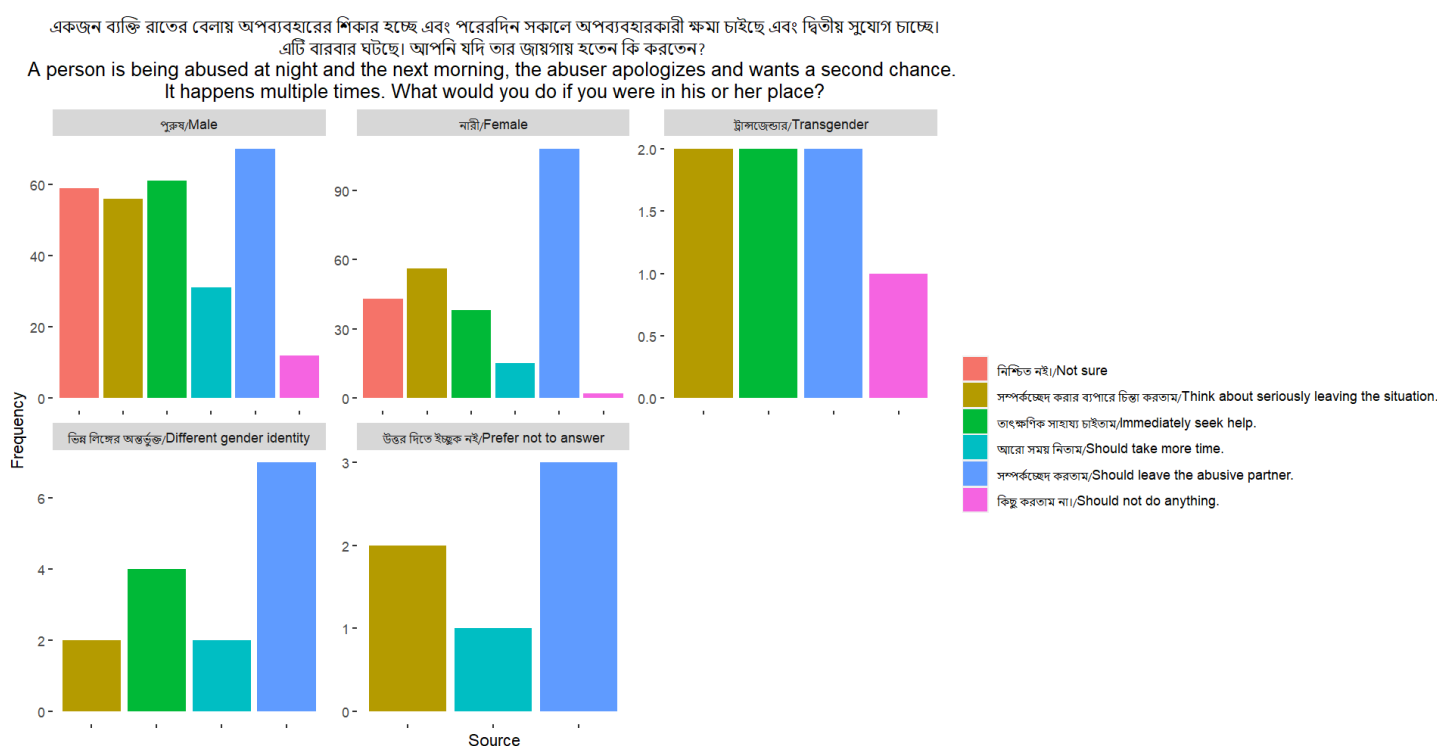


Majority of the participants ‘under the age of 18’ and who ‘do not wish to disclose their age’ are not sure about this.

Majority of the participants over the age of 20 think they should leave the abusive partner.

Respondents who are between the age of 18 and 20 are almost evenly split between ‘not sure’ and ‘should leave the abusive partner’

The range of varied responses shows the complexities of the decision making process in abusive relationships.



Transgender respondents are almost evenly split among ‘Not Sure’, ‘Immediately Seek Help’, and ‘Think About Seriously Leaving The Situation’. While the majority of the respondents of other genders are not sure about this.

Majority (59.56%) of the participants think that mental health is as important as physical health

Majority of the people in all the age groups except those who ‘Do not wish to specify’ their age believe mental health is as important as physical health.

While the majority of those of ‘Do not wish to specify’ their age believe it’s more important.

Majority of the people in all the gender groups except those from ‘Different Gender Identity’ believe mental health is as important as physical health.

While those from ‘Different Gender Identity’ believe it’s more important.

Majority (56.82%) of the participants find taking professional therapy a useful tool for helping them to stay healthy.

Programmatic recommendation:

Our survey demonstrates a significant knowledge gap in skills for health and well-being (communication, refusal and negotiation skills); suggesting a pressing demand for thorough and specific education. Here are some important topics we need to cover:

- **Abuse prevention and intervention events:** Our survey shows that 32.81% of respondents believe that they should leave an abusive partner, but many others are not sure or would stay in the situation. This indicates a need for events that can help individuals understand the dynamics of abusive relationships and the options available for them. These events should also address the emotional and psychological aspects of abuse, such as anxiety and depression, which affect 55.44% of respondents often.

(Aligns with **Pathway 2: Public Support** and **Pathway 3: Policy Advocacy**)

- **Legal rights awareness events:** The data shows that transgender respondents are more likely to seek help or leave an abusive situation than other genders. This suggests potential barriers or discrimination in accessing legal rights and resources, possibly heightened by concerns about societal stigma. Legal rights awareness events can help inform them of their rights and the support they can receive from legal professionals and organizations. Ensuring collaboration with social influencers is crucial in mitigating concerns about societal stigma, fostering an environment where survivors feel empowered to seek assistance without fear of judgment or discrimination.

(Aligns with **Pathway 2: Public Support** and **Pathway 3: Policy Advocacy**)

- **Communication skills workshops:** The data shows that 59.41% of respondents believe effective communication is a main aspect of a healthy relationship. However, 53.88% of respondents also admit that they sometimes get angry or upset when someone says no to them. This indicates a need for initiatives that can help individuals improve their communication skills and learn how to handle rejection gracefully. These workshops can also teach individuals how to communicate their sexual health boundaries and preferences, as 75.6% of respondents claim they can do so effectively.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Advocacy for Mental Health Policies:** Mental health is a critical aspect of recovery for survivors. Our data shows that 59.56% of respondents think that mental health is as important as physical health, and only 56.82% find taking professional therapy a useful tool for staying healthy.. This highlights the importance of advocating for policies that address mental health concerns and support survivors of abusive relationships. Such policies can ensure that individuals have access to affordable and quality mental health services and resources.

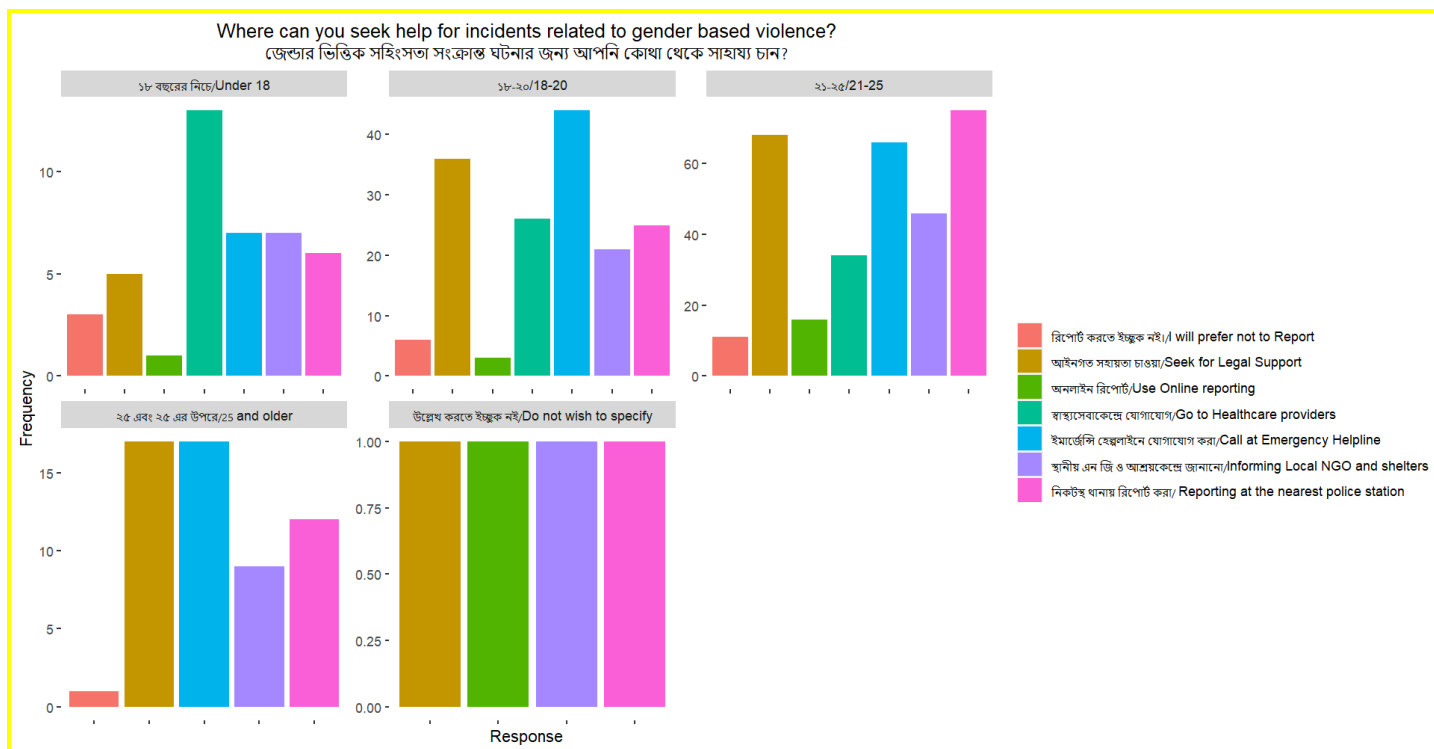
(Aligns with **Pathway 3: Policy Advocacy**)

Human Rights and Sexuality (Local and/or national laws and international agreements address human rights that impact sexual and reproductive health).

The respondents seem well versed as to where they may seek help for incidents related to gender based violence. However, a very small percentage of the respondents do not wish to report such incidents, which is alarming. Steps need to be taken so that no GBV goes unreported.

Key Findings

Majority (23.14%) of the participants responded that they should call an emergency helpline to seek help for incidents related to gender based violence.



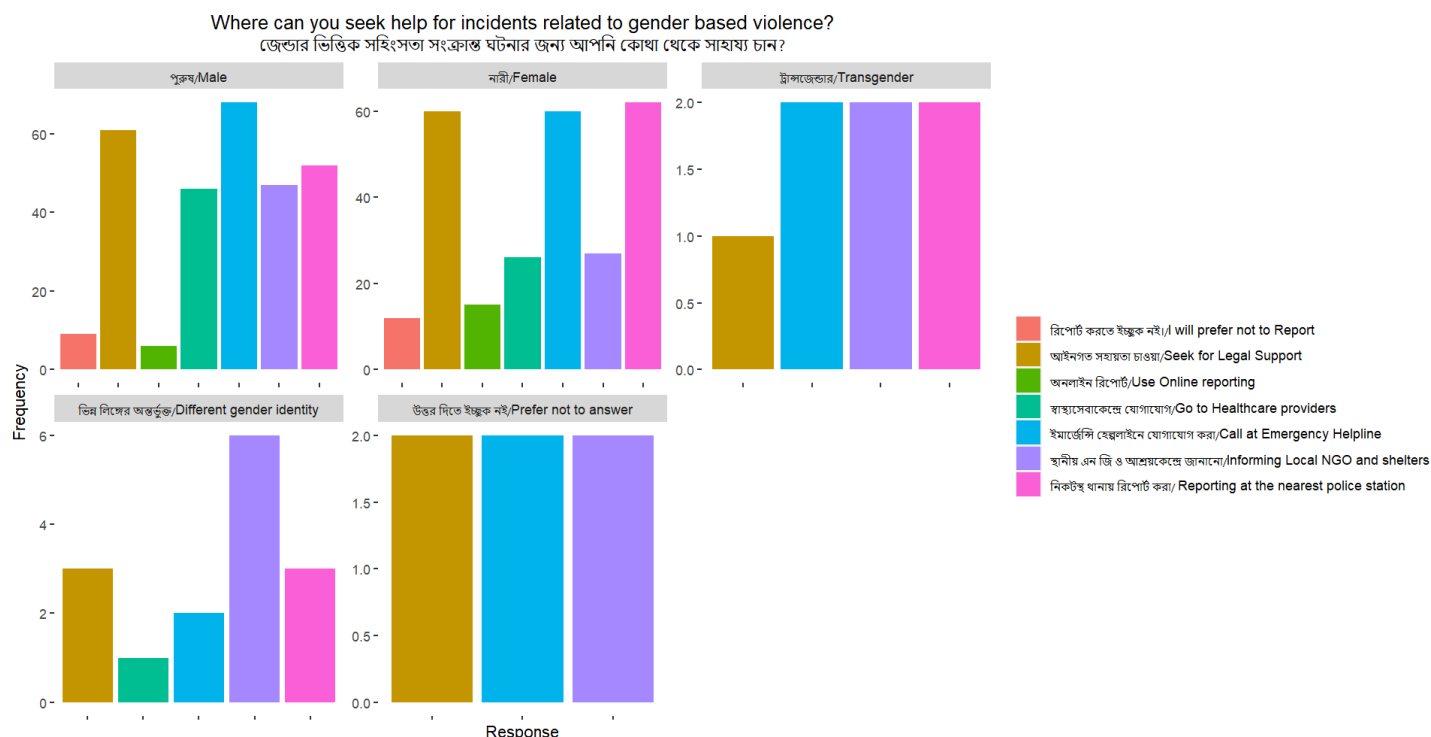
Majority of the respondents below the age of 18 think they should go to healthcare providers to seek help for incidents related to gender based violence.

Majority of the respondents between the ages 18 and 20 believe they should call an emergency helpline to seek help for incidents related to gender based violence.

Majority of the respondents between the ages 21 and 25 think they should report at the nearest police station to seek help for incidents related to gender based violence.

Majority of the respondents between the age 21 and 25 are evenly split between ‘Seek legal Support’ and ‘Call emergency helpline’ to seek help for incidents related to gender based violence.

We can highlight the key findings based on age based variance, gender based preferences and general awareness. This data indicates we need tailored awareness campaigns considering the diverse perspectives



Majority of the Male respondents believe they should call an emergency helpline to seek help for incidents related to gender based violence.

Majority of the female respondents think they should report at the nearest police station to seek help for incidents related to gender based violence.

Majority of the respondents with 'Different gender think they should inform local NGO and shelters to seek help for incidents related to gender based violence.

Majority of the transgender respondents however are evenly split, thinking that they should inform local NGO and shelters, report at the nearest police station, and go to healthcare providers to seek help for incidents related to gender based violence.

Majority (78.1%) of the respondents are aware of the services Bangladesh has related to any SRHR related issues.

The data reveals gender-specific preferences in seeking help for gender-based violence incidents, highlighting the need for diverse support channels. The findings emphasize the importance of tailored support systems and widespread awareness of available services.

Programmatic recommendation:

The findings from our survey reveal a concerning lack of understanding about human rights and sexuality; indicating an urgent need for comprehensive and targeted education. Here are several key areas that require immediate attention:

- **Developing Age, Gender, and Awareness Level-Specific Campaigns:** The data shows that different age groups, genders, and awareness levels have different preferences and needs when it comes to seeking help for gender-based violence incidents. For example, respondents below the age of 18 are more likely to go to healthcare providers, while respondents between the ages 21 and 25 are more likely to report at the nearest police station or seek legal support. This indicates a need to develop campaigns that consider the unique needs and experiences of different age groups, genders, and awareness levels. These campaigns should be tailored to address the specific concerns and challenges faced by each group, ensuring that everyone has access to relevant and effective resources.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Building Trust in emergency services:** The survey indicates that only 23.14% of respondents would consider using an emergency helpline in instances of Gender-Based Violence (GBV), highlighting a significant need to build more trust in these services. To address this, we propose implementing several strategies: ensuring transparency in helpline operations, providing regular and rigorous training for helpline staff, conducting public awareness campaigns about the helplines, collaborating with trusted organizations like healthcare providers and NGOs to endorse the helpline, and establishing a robust feedback mechanism. These measures aim to enhance the reliability of emergency services for GBV, encouraging more individuals to seek help when needed, and contributing to a safer and more supportive society.

(Aligns with **Pathway 2: Public Support**)

- **Identifying the information gap for younger respondents:** Focus on respondents below the age of 18 as our survey data shows that respondents below the age of 18 are less aware of the services available for SRHR related issues than other age groups; and identify gaps in their understanding of consent and healthy relationships. We need to develop alternative support options that are accessible and appealing to this age group. This could include online resources, peer-led workshops, or school-based programs.

(Aligns with **Pathway 1: Information and Education** and **Pathway 2: Public Support**)

- **Targeted services for age 21-15:** The data shows that respondents between the ages 21 and 25 have diverse preferences when it comes to seeking help for gender-based violence incidents. Some prefer to call an emergency helpline, while others prefer to seek legal support. This highlights the need for comprehensive support systems that cater to these preferences. This could involve a mix of online resources, community-based services, and professional counseling to ensure that everyone within this age group has access to the support they need.

(Aligns with **Pathway 2: Public Support**)

- **Specialized support system for transgender communities:** The data shows that transgender respondents are more likely to seek help from local NGOs and shelters, report at the nearest police station, or go to healthcare providers than other genders. This suggests that they face unique challenges when it comes to consent and healthy relationships, and may not feel comfortable or safe with other support channels. This emphasizes the need for a specialized support system that provides resources and services that are specifically designed to address these challenges. This system should provide resources and services that are specifically designed to address these challenges.

(Aligns with **Pathway 2: Public Support** and **Pathway 3: Policy Advocacy**)

Limitations of the Survey

1. **Limited Sample Size:** The 579 respondents in the survey may restrict the findings' applicability to the larger group of university students. The specific backgrounds and experiences of this study might not fully represent the diversity within the student community.
2. **Limited Geographical Data:** Our survey was conducted utilizing online platforms. As the survey form was circulated openly, we did not have control over the geographical diversity of respondents, which led to less geographical diversity. This limitation may have resulted in a representation bias among the respondents.
3. **The Taboo Nature of SRHR:** Because SRHR is such a sensitive topic, societal stigma around it may cause underreporting or biased responses. Societal stigma such as fear of judgment, societal expectations, privacy concerns, religious and moral aspects might lead respondents to withhold their opinions, while impacting the accuracy of the data collection process.
4. **Potential for Social Desirability Bias:** Due to the self reported nature of survey responses, there is a possibility of social desirability bias. The respondents may prefer to provide answers they think are socially acceptable rather than sharing their own opinions. Respondents may consciously or unconsciously be mindful of answering controversial questions to avoid or minimize judgements.
5. **Limited Representation:** We cannot say that our survey represents the university students of Bangladesh since students self-select into the survey; therefore, we may represent certain types of university/college students more than others. Students who do not find the survey topics relevant or interesting, might choose not to participate. Such under-representation of specific groups can draw skewed datasets.

6. **Temporal Bias:** This survey captures views of the respondents at a specific moment, potentially missing evolving societal attitudes towards SRHR. These opinions can change over time and this timeframe may not entirely capture the dynamic nature of these issues.
7. **Inability to Clarify Ambiguities:** Limited opportunity to clarify questions online may lead to misinterpretation of survey items, impacting response accuracy. The participants might miss out on the chance to seek clarification on unclear aspects.

Key Takeaways and recommendations:

In pursuit of our objective to develop impactful strategies that align with the study's insights and effectively address the identified needs in the survey, let's explore the following major takeaways and recommendations-

Primary Information Sources on SRHR

- **Finding:** 45.6% of respondents identified friends and family as their primary source of SRHR information.
- **Recommendation:** Focus on developing family-oriented initiatives to educate on SRHR. This can involve workshops, seminars, or educational materials designed specifically for families from diverse backgrounds.

- **Finding:** Majority of participants aged 25 or below received sex education at school, while those aged 25 and above mostly did not.
- **Recommendation:** Implement ongoing and inclusive comprehensive sex education programs addressing knowledge gaps among all age groups. We need to ensure the contents cover a wide range of SRHR topics and that it is tailored to be age appropriate and relevant.

- **Finding:** Participants from government educational institutions mostly responded that they have received sex education in school, while participants from private institutions mostly responded in the negative.
- **Recommendation:** Bridge the gap in sex education by updating the curriculum in private institutions or providing adequate training to teachers. We will need policy advocacy, collaboration with the educational board and frequent training initiatives.

Understanding Consent and Healthy Relationships

- **Finding:** Majority (69.26%) of the participants strongly disagree with the statement that a girl's clothing choices suggest she is seeking or open to sexual activity. However 51.8% of respondents believe that any physical reaction implies consent.
- **Recommendation:** Launch a targeted consent education campaign focusing on the nuances of consent that will reinforce positive attitudes towards consent and challenge harmful stereotypes. Implement awareness campaigns to debunk myths related to clothing choices and consent. This could involve interactive workshops, online resources, and awareness campaigns.
- **Finding:** Views on consent vary among different gender identities. While most male participants somewhat agree with physical reactions equating to consent, most transgender respondents 'somewhat disagree.' Female and 'different gender identity' respondents strongly disagree.
- **Recommendation:** Implement inclusive education that recognizes and respects diverse perspectives on consent. Address the specific needs and concerns of different gender identities.
- **Finding:** Our survey shows that younger participants often mistake jealousy and possessiveness as signs of strong love.
- **Recommendation:** There is a pressing need for programs that can enlighten the youth about the nuances of a healthy relationship, and aid them in differentiating between love and dominance.
- **Finding:** Majority (40.76%) of the participants strongly agree that if an individual isn't resisting or vocalizing objections, it can be assumed that they are agreeing for sexual relationships.
- **Recommendation:** Emphasize the importance of explicit communication in intimate relationships. Address the complexities of non-verbal communication in consent education programs.

Gender Equality, Bias, and Stereotypes

- **Finding:** 81.86% of respondents feel that the husband or husband's family have more dominance in decision-making in Bangladeshi marriages.
 - **Recommendation:** Arrange interactive workshops, partnering with schools and universities, to discuss and debunk biased norms and gender stereotypes.
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- **Finding:** Majority (52.68%) of the respondents did not hear phrases like “act like a boy” or “act like a girl” at their home from their family.
 - **Recommendation:** While the majority of male and female respondents did not hear such phrases, a significant portion of other genders did. Organize gender sensitization events to challenge traditional gender norms and empower individuals of all genders.
-
- **Finding:** Majority (58.2%) of the participants strongly agree that some textbook chapters in Bangladesh depict men in leadership roles and women in passive or domestic roles, contributing to gender bias.
 - **Recommendation:** Educational materials should be reviewed and revised to eliminate gender stereotypes. The RHRN coalition staff, youth groups, youth advisory committee, and university clubs should be trained through capacity-strengthening sessions on the policy development process, influencing local governance, and writing policy memos, among other things. These sessions will aid in policy formulation and pave the way for conducting awareness campaigns. These campaigns aim to raise awareness about the impact of such depictions and promote gender equality in educational content.
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- **Finding:** Majority (43.86%) of the participants think premarital sex is okay as long as it has consent from both parties.
 - **Recommendation:** Educate individuals about the concept of consent, particularly in premarital relationships. Launch awareness campaigns emphasizing the importance of clear, enthusiastic, and ongoing consent in intimate relationships.

Basic Knowledge of SRHR and STDs

- **Finding:** 72.72% of respondents believe they should approach a doctor for information about contraceptives, pregnancy, STDs, or sexual health.

- **Recommendation:** Develop comprehensive educational resources on these topics and promote accessible healthcare services. Collaborate with healthcare professionals to ensure the information provided is accurate and up-to-date.

Skills for Health and Well-being

- **Finding:** 55.44% of respondents often feel depressed or anxious.
- **Recommendation:** Initiate mental health awareness campaigns, offering age-appropriate resources and professional support systems. Explore online platforms for wider reach and accessibility.
- **Finding:** Majority (59.41%) believe effective communication, listening to your partner, being able to speak for yourself, taking a stand for yourself without the fear of judgment, mutual trust and respect, and setting personal space and boundaries are the main aspects of a healthy relationship.
- **Recommendation:** Conduct communication skills workshops to help individuals improve their ability to express themselves effectively, understand the importance of mutual trust and respect, and set personal boundaries in relationships.
- **Finding:** Majority (32.81%) of the respondents believe that they should leave an abusive partner, but many others are not sure or would stay in the situation. The responses vary significantly across age groups, genders, and gender identities.
- **Recommendation:** Organize abuse prevention and intervention events to help individuals understand the dynamics of abusive relationships. Offer support for those facing anxiety or depression due to abuse. Additionally, conduct legal rights awareness events, ensuring collaboration with influencers to address potential social barriers or discrimination faced by individuals in accessing legal resources.
- **Finding:** Majority (53.88%) responded that when someone says NO to them, it sometimes makes them angry or upset.
- **Recommendation:** Conduct workshops to improve communication skills and teach individuals how to handle rejection gracefully. Emphasize the importance of effective communication in sexual health discussions, with a focus on refusal and negotiation skills.

Human Rights and Sexuality

- **Finding:** The study shows that 23.14% of respondents believe they should call an emergency helpline for Gender-Based Violence (GBV) incidents.
- **Recommendation:** Consider the diverse preferences in seeking help for GBV. Collaborate with helplines, healthcare providers, and NGOs to provide well-rounded assistance. This could involve awareness campaigns, support services, and legal aid.
- **Finding:** Different genders have distinct preferences in seeking help for gender-based violence. Males prefer calling an emergency helpline, females lean towards reporting at the nearest police station, those with 'Different' gender prefer informing local NGOs and shelters, and transgender respondents show an even split among multiple options.
- **Recommendation:** Recognize and address gender-specific preferences. Develop specialized support systems for transgender communities, ensuring resources and services that specifically address their unique challenges in consent and healthy relationships.
- **Finding:** Majority (78.1%) of the respondents are aware of the services Bangladesh has related to any SRHR related issues.
- **Recommendation:** Continue to promote awareness of services related to SRHR issues. Identify information gaps for younger respondents and develop strategies, such as online resources, peer-led workshops, or school-based programs, to enhance understanding among this age group.

Conclusion

This survey serves as a comprehensive overview of the understanding of Sexual and Reproductive Health and Rights (SRHR) among university students, highlighting the influential role of friends and family. While the participants exhibited a robust foundational comprehension of SRHR, the identified gaps underscore the need for focused attention, particularly in the areas of communication skills and misconceptions about consent. These findings emphasize the necessity for continuous education and targeted interventions to enhance overall awareness.

The report advocates for continuous collaboration in the pursuit of all-encompassing SRHR education, promoting open discussions and policy changes. As we progress, Youth Policy Forum (YPF) advocates for a strategic emphasis on future research endeavors, promoting longitudinal studies for an enriched understanding over an extended period. Broadening the demographic spectrum in subsequent surveys will yield more profound insights into the unique perspectives of diverse groups. Furthermore, qualitative investigations can reveal nuanced viewpoints, thereby contributing to the development of more effective educational strategies.

